

Acute Bacterial Bronchitis (chest service)

Patient Group Direction, version 004, issued 9 September 2026. Three arms: doxycycline, amoxicillin, clarithromycin.

Which arm applies

ARM 1, DOXYCYCLINE: adults 18 and over. First line.

ARM 2, AMOXICILLIN: patients aged 12 to 17; pregnant patients of any age; AND adults 18 and over for whom doxycycline is unsuitable and who are NOT penicillin-allergic.

ARM 3, CLARITHROMYCIN: patients aged 12 and over who are PENICILLIN-ALLERGIC and for whom the first-line agent for their circumstances is unsuitable or unavailable.

The first-line agent means doxycycline in adults, and amoxicillin at 12 to 17 and in pregnancy. A penicillin allergy makes amoxicillin unsuitable by itself, so a penicillin-allergic 12 to 17 year old goes straight to Arm 3.

Before anything else

- **MOST PATIENTS PRESENTING WITH AN ACUTE COUGH DO NOT NEED AN ANTIBIOTIC.** The inclusion criteria are deliberately narrow, and a cough alone may take three weeks to settle.
- Every observation in Appendix 1 must be measured and recorded before any supply, and the CRB score calculated. If any threshold is breached, refer.
- One course per episode. A second course is not authorised under this PGD.

Summary of NICE guidance for acute cough and acute bronchitis

NICE NG120, Cough (acute): antimicrobial prescribing; NICE Clinical Knowledge Summaries, Chest infections in adults. Summarised 9 September 2026.

Overview

Acute cough is usually caused by a viral upper respiratory tract infection or by acute bronchitis, and is usually SELF LIMITING.

A cough from acute bronchitis commonly lasts around three weeks, and that duration alone is not a reason to treat.

Purulent sputum on its own does not indicate a bacterial infection needing an antibiotic.

What NICE advises

- DO NOT routinely offer an antibiotic for acute cough in an otherwise healthy adult. Give advice on the expected duration and on self care.
- CONSIDER an antibiotic where the person is systemically very unwell, or has symptoms and signs suggesting a serious illness or complication such as pneumonia.
- CONSIDER an antibiotic where the person is at higher risk of complications: significant comorbidity, immunosuppression, or being older and frail.
- DOXYCYCLINE is first choice in adults. Amoxicillin is a first-choice alternative. Clarithromycin or erythromycin are alternatives where the first choice is unsuitable, erythromycin being the option used in pregnancy.

Recognising pneumonia rather than bronchitis

Focal chest signs, such as a dull percussion note, bronchial breathing, or coarse crackles that do not clear on coughing, together with any systemic feature, suggest pneumonia.

Pneumonia is not covered by this PGD and needs assessment the same day.

CRB-65 is a severity score for community acquired pneumonia. This PGD uses it as a safety net, deliberately WITHOUT the age point, and Appendix 1 explains why.

How this PGD applies that guidance

The inclusion criteria are deliberately narrow: purulent sputum PLUS either a higher-risk comorbidity or symptoms beyond 14 days.

Most people presenting with an acute cough will not meet them, and that is the guidance working rather than the service failing.

Every observation in Appendix 1 must be measured before any supply, and anything outside them refers.

Arm 1. Doxycycline, first line, adults

Patient Group Direction for the supply of doxycycline 100mg capsules for acute bacterial bronchitis in adults aged 18 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording oxygen saturation on a validated pulse oximeter, respiratory rate, pulse, blood pressure and temperature, and in applying the thresholds in Appendix 1.
- Must be able to calculate and apply the CRB score used in this service, and must understand that the age point of CRB-65 is deliberately NOT applied here, and why.
- Must know which arm applies to which patient. The rule is stated once at the front of this document and repeated in each arm.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Acute bacterial bronchitis in adults aged 18 and over meeting the inclusion criteria below. First-line agent for adults.
Which arm applies	ARM 1, DOXYCYCLINE: adults 18 and over. First line. ARM 2, AMOXICILLIN: patients aged 12 to 17; pregnant patients of any age; AND adults 18 and over for whom doxycycline is unsuitable and who are NOT penicillin-allergic. ARM 3, CLARITHROMYCIN: patients aged 12 and over who are PENICILLIN-ALLERGIC and for whom the first-line agent for their circumstances is unsuitable or unavailable. The first-line agent means doxycycline in adults, and amoxicillin at 12 to 17 and in pregnancy. A penicillin allergy makes amoxicillin unsuitable by itself, so a penicillin-allergic 12 to 17 year old goes straight to Arm 3.
Inclusion criteria	• Aged 18 years and over. • PURULENT SPUTUM (yellow or green), AND EITHER a higher-risk comorbidity (chronic lung disease including COPD and asthma, heart failure, diabetes, chronic kidney or liver disease, immunosuppression, or age 65 and over) OR symptoms persisting beyond 14 days. • All observations within the thresholds in Appendix 1, and a CRB score of 0. The CRB score used in this service does NOT include the age point: see Appendix 1. • Valid informed consent given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present.

	● SUSPECTED PNEUMONIA: focal chest signs (dull percussion note, bronchial breathing, coarse crackles that do not clear with coughing) with any systemic feature. ● Any observation outside the thresholds in Appendix 1, or a CRB score of 1 or more. ● SpO2 below 94% on air at rest. ● Severely systemically unwell, however the observations read. If the patient looks unwell to you, refer. ● Haemoptysis, or a cough lasting more than 3 weeks in a current or former smoker. ● Suspected pulmonary embolism, heart failure or lung cancer. ● An antibiotic already taken for this episode. One course per episode. ● Under 18 years of age. Use the amoxicillin arm. ● Pregnancy or breastfeeding. Doxycycline is contraindicated; use the amoxicillin arm. ● Hypersensitivity to doxycycline or other tetracyclines. ● Known severe hepatic impairment. ● CONCURRENT ISOTRETINOIN. Doxycycline with a retinoid risks benign intracranial hypertension. Refer.
Cautions	Photosensitivity: advise sun protection. Separate from antacids, iron and dairy by at least 2 hours. Potentiates warfarin; where the patient is anticoagulated, refer rather than supply. The same finding is an EXCLUSION in the Skin and Soft Tissue Infection PGD. The practical outcome is identical in both services: do not supply, refer. The difference in wording is noted here because staff move between the two within a shift.
Actions if excluded or declines	Explain why an antibiotic cannot be supplied, and say plainly that most acute coughs do not need one and settle on their own. Refer same-day where any Appendix 1 threshold is breached, the CRB score is 1 or more, or pneumonia is suspected. Give self-care and safety-netting advice, including that a cough alone may take three weeks to settle. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding such as a low SpO2.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given, and where the patient is under 16, from whom, or the basis of the Gillick assessment. ● Patient name, address, date of birth, and the GP with whom they are registered. ● The full set of observations from Appendix 1, including SpO2, respiratory rate, pulse, blood pressure and temperature, and the CRB score. ● Which inclusion feature was met: purulent sputum plus comorbidity, or purulent sputum plus duration. Where the comorbidity was age 65 and over, that the lower referral threshold for that group was considered. ● WHICH ARM was used and why, naming the reason the first-line agent was not used where a second-line arm was chosen. ● Name and registration number of the healthcare professional supplying. ● Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. ● Advice given, including advice given if excluded or declining treatment. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD.

	Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday.
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The medicine

Name, form and strength	Doxycycline 100mg capsules.
Legal category	POM.
Route and method	Oral. Swallow whole with plenty of water, sitting or standing, and well before lying down, to avoid oesophageal irritation.
Dose and frequency	200mg on day 1 as a single dose, then 100mg once daily for a further 4 days. Total 5 days.
Quantity to be supplied	6 capsules. Two on day 1, then one daily for 4 days.
Maximum treatment period	5 days. One course per episode. A second course is not authorised under this PGD; refer.
Adverse effects	Common: nausea, diarrhoea, photosensitivity. Uncommon: oesophageal irritation or ulceration, rash. Rare: Clostridioides difficile infection, hepatotoxicity, benign intracranial hypertension.
Storage	Store below 25C.

Information for the patient

Written information	Supply the doxycycline patient information leaflet.
Counselling	• Take with plenty of water, sitting or standing up, and do not lie down for 30 minutes afterwards. • Avoid antacids, indigestion remedies, iron tablets and milk within 2 hours of a dose. • You may burn more easily in the sun. Use sun protection. • Seek help the same day if you become breathless, develop chest pain, cough blood, or feel much worse at any point. Do not wait to finish the course. • Come back if you are no better 5 to 7 days after finishing.
Follow-up	Seek advice if breathlessness, chest pain or fever develop, if symptoms worsen at any point, or if there is no improvement within 5 to 7 days of finishing the course. A cough alone may take three weeks to settle and is not by itself a reason to return.

Arm 2. Amoxicillin, 12 to 17 years, pregnancy, and adults where doxycycline is unsuitable

Patient Group Direction for the supply of amoxicillin 500mg capsules for acute bacterial bronchitis in patients aged 12 to 17 years, in pregnancy, and in adults aged 18 and over for whom doxycycline is unsuitable and who are not penicillin-allergic.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording oxygen saturation on a validated pulse oximeter, respiratory rate, pulse, blood pressure and temperature, and in applying the thresholds in Appendix 1.
- Must be able to calculate and apply the CRB score used in this service, and must understand that the age point of CRB-65 is deliberately NOT applied here, and why.
- Must know which arm applies to which patient. The rule is stated once at the front of this document and repeated in each arm.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Acute bacterial bronchitis in patients aged 12 to 17 years; in pregnancy; and in non-penicillin-allergic adults for whom doxycycline is unsuitable. Amoxicillin is a NICE first-choice alternative in acute bronchitis.
Which arm applies	ARM 1, DOXYCYCLINE: adults 18 and over. First line. ARM 2, AMOXICILLIN: patients aged 12 to 17; pregnant patients of any age; AND adults 18 and over for whom doxycycline is unsuitable and who are NOT penicillin-allergic. ARM 3, CLARITHROMYCIN: patients aged 12 and over who are PENICILLIN-ALLERGIC and for whom the first-line agent for their circumstances is unsuitable or unavailable. The first-line agent means doxycycline in adults, and amoxicillin at 12 to 17 and in pregnancy. A penicillin allergy makes amoxicillin unsuitable by itself, so a penicillin-allergic 12 to 17 year old goes straight to Arm 3.
Inclusion criteria	• Aged 12 years and over. • AND ONE OF: aged 12 to 17; OR pregnant; OR aged 18 and over with a recorded reason why doxycycline is unsuitable. • NOT penicillin-allergic. Where the patient is, use Arm 3. • PURULENT SPUTUM (yellow or green), AND EITHER a higher-risk comorbidity (chronic lung disease including COPD and asthma, heart failure, diabetes, chronic kidney or liver disease, immunosuppression, or age 65 and over) OR symptoms persisting beyond 14 days. • All observations within the thresholds in Appendix 1, and a CRB score of 0. The CRB score used in this service does NOT include the age point: see Appendix 1.

	• Valid informed consent given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • SUSPECTED PNEUMONIA: focal chest signs (dull percussion note, bronchial breathing, coarse crackles that do not clear with coughing) with any systemic feature. • Any observation outside the thresholds in Appendix 1, or a CRB score of 1 or more. • SpO2 below 94% on air at rest. • Severely systemically unwell, however the observations read. If the patient looks unwell to you, refer. • Haemoptysis, or a cough lasting more than 3 weeks in a current or former smoker. • Suspected pulmonary embolism, heart failure or lung cancer. • An antibiotic already taken for this episode. One course per episode. • Under 12 years of age. • Known penicillin or beta-lactam allergy, or any history of cephalosporin allergy. Use Arm 3. • Infectious mononucleosis or acute lymphoblastic leukaemia, because of the risk of a widespread rash. • Known significant renal impairment. Refer.
Cautions	Amoxicillin may be supplied in pregnancy and in breastfeeding. It is the usual choice in pregnancy where an antibiotic is indicated for this condition. Diarrhoea is common. Advise the patient to seek advice for severe or bloody diarrhoea, which may indicate <i>Clostridioides difficile</i> infection. A non-allergic maculopapular rash is common with amoxicillin and is not the same as allergy, but any rash should be reviewed.
Actions if excluded or declines	Explain why an antibiotic cannot be supplied, and say plainly that most acute coughs do not need one and settle on their own. Refer same-day where any Appendix 1 threshold is breached, the CRB score is 1 or more, or pneumonia is suspected. Give self-care and safety-netting advice, including that a cough alone may take three weeks to settle. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding such as a low SpO2.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and where the patient is under 16, from whom, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • The full set of observations from Appendix 1, including SpO2, respiratory rate, pulse, blood pressure and temperature, and the CRB score. • Which inclusion feature was met: purulent sputum plus comorbidity, or purulent sputum plus duration. Where the comorbidity was age 65 and over, that the lower referral threshold for that group was considered. • WHICH ARM was used and why, naming the reason the first-line agent was not used where a second-line arm was chosen. • Where the patient is an adult, the specific reason doxycycline was unsuitable. • Name and registration number of the healthcare professional

	supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday.
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The medicine

Name, form and strength	Amoxicillin 500mg capsules.
Legal category	POM.
Route and method	Oral. Swallow whole with water, with or without food.
Dose and frequency	500mg three times daily for 5 days.
Quantity to be supplied	15 capsules. Supply the whole course.
Maximum treatment period	5 days. One course per episode; a second course is not authorised.
Adverse effects	Common: diarrhoea, nausea, rash. Uncommon: vomiting, abdominal discomfort, oral candidiasis. Rare: allergic reactions. Very rare: anaphylaxis, Stevens-Johnson syndrome, hepatitis and cholestatic jaundice, Clostridioides difficile colitis.
Storage	Store below 25C in the original container.

Information for the patient

Written information	Supply the amoxicillin patient information leaflet.
Counselling	• One capsule three times a day, every 8 hours, and finish the course. • Seek help the same day if you become breathless, develop chest pain, cough blood, or feel much worse. Do not wait to finish the course. • Some diarrhoea is common. Get advice if it is severe or bloody. • A rash with this antibiotic is usually not an allergy, but get it checked, and seek urgent help for wheeze or swelling of the lips or tongue. • Come back if you are no better 5 to 7 days after finishing.
Follow-up	As Arm 1.

Arm 3. Clarithromycin, penicillin allergy

Patient Group Direction for the supply of clarithromycin 250mg tablets for acute bacterial bronchitis in patients aged 12 years and over who are penicillin-allergic and for whom the first-line agent for their circumstances is unsuitable or unavailable.

One rule, stated the same way in the heading, the indication and the inclusion criteria.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. None of these medicines is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording oxygen saturation on a validated pulse oximeter, respiratory rate, pulse, blood pressure and temperature, and in applying the thresholds in Appendix 1.
- Must be able to calculate and apply the CRB score used in this service, and must understand that the age point of CRB-65 is deliberately NOT applied here, and why.
- Must know which arm applies to which patient. The rule is stated once at the front of this document and repeated in each arm.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Acute bacterial bronchitis in patients aged 12 years and over who are PENICILLIN-ALLERGIC and for whom the first-line agent for their circumstances is unsuitable or unavailable. The first-line agent means doxycycline in adults, and amoxicillin at 12 to 17 and in pregnancy; a penicillin allergy makes amoxicillin unsuitable by itself.
Which arm applies	ARM 1, DOXYCYCLINE: adults 18 and over. First line. ARM 2, AMOXICILLIN: patients aged 12 to 17; pregnant patients of any age; AND adults 18 and over for whom doxycycline is unsuitable and who are NOT penicillin-allergic. ARM 3, CLARITHROMYCIN: patients aged 12 and over who are PENICILLIN-ALLERGIC and for whom the first-line agent for their circumstances is unsuitable or unavailable. The first-line agent means doxycycline in adults, and amoxicillin at 12 to 17 and in pregnancy. A penicillin allergy makes amoxicillin unsuitable by itself, so a penicillin-allergic 12 to 17 year old goes straight to Arm 3.
Inclusion criteria	• Aged 12 years and over. • PENICILLIN-ALLERGIC. • AND the first-line agent for this patient is unsuitable or unavailable, with the reason recorded. For a patient aged 12 to 17, or a pregnant patient, the penicillin allergy alone satisfies this, because amoxicillin is their first-line agent. • PURULENT SPUTUM (yellow or green), AND EITHER a higher-risk comorbidity (chronic lung disease including COPD and asthma, heart failure, diabetes, chronic kidney or liver disease, immunosuppression, or age 65 and over) OR symptoms persisting beyond 14 days.

	• All observations within the thresholds in Appendix 1, and a CRB score of 0. The CRB score used in this service does NOT include the age point: see Appendix 1. • Valid informed consent given. Where the patient is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis recorded. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • SUSPECTED PNEUMONIA: focal chest signs (dull percussion note, bronchial breathing, coarse crackles that do not clear with coughing) with any systemic feature. • Any observation outside the thresholds in Appendix 1, or a CRB score of 1 or more. • SpO2 below 94% on air at rest. • Severely systemically unwell, however the observations read. If the patient looks unwell to you, refer. • Haemoptysis, or a cough lasting more than 3 weeks in a current or former smoker. • Suspected pulmonary embolism, heart failure or lung cancer. • An antibiotic already taken for this episode. One course per episode. • Under 12 years of age. • Pregnancy or breastfeeding. Refer. • Hypersensitivity to macrolides. • KNOWN RENAL IMPAIRMENT with a creatinine clearance below 30 mL/min, or renal impairment of unknown severity where there is reason to suspect it is significant. Refer. • Concurrent ergot alkaloids, oral midazolam, lomitapide, astemizole, cisapride, domperidone, pimozide, terfenadine, ticagrelor, ivabradine, ranolazine, simvastatin or lovastatin. • Known QT prolongation, or concurrent QT-prolonging medicines. • Known electrolyte disturbance, or severe hepatic impairment. • Taking warfarin or a DOAC. Refer.
Cautions	Colchicine: risk of toxicity. Refer rather than supply. Advise on the potential for dizziness, vertigo and taste disturbance. Ask about renal function before supply and record the answer. Where the patient does not know and there is no reason to suspect impairment, supply and record that you asked.
Actions if excluded or declines	Explain why an antibiotic cannot be supplied, and say plainly that most acute coughs do not need one and settle on their own. Refer same-day where any Appendix 1 threshold is breached, the CRB score is 1 or more, or pneumonia is suspected. Give self-care and safety-netting advice, including that a cough alone may take three weeks to settle. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding such as a low SpO2.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given, and where the patient is under 16, from whom, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • The full set of observations from Appendix 1, including SpO2, respiratory rate, pulse, blood pressure and temperature, and the CRB score. • Which inclusion feature was met: purulent sputum plus comorbidity, or purulent sputum plus duration. Where the

	comorbidity was age 65 and over, that the lower referral threshold for that group was considered. ● WHICH ARM was used and why, naming the reason the first-line agent was not used where a second-line arm was chosen. ● The penicillin allergy history in the patient own terms. ● That renal function was asked about, and the answer given. ● Name and registration number of the healthcare professional supplying. ● Name of the medicine, date of supply, dose, form, route and quantity supplied, with batch number and expiry date. ● Advice given, including advice given if excluded or declining treatment. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday.
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The medicine

Name, form and strength	Clarithromycin 250mg tablets.
Legal category	POM.
Route and method	Oral, with or without food.
Dose and frequency	250mg twice daily for 5 days.
Quantity to be supplied	10 tablets.
Maximum treatment period	5 days. One course per episode; a second course is not authorised.
Adverse effects	Common: abdominal pain, nausea, diarrhoea, taste disturbance, headache. Uncommon: QT prolongation, hepatic dysfunction, dizziness, vertigo. Rare: Clostridioides difficile infection, Stevens-Johnson syndrome.
Storage	Store below 25C.

Information for the patient

Written information	Supply the clarithromycin patient information leaflet.
Counselling	● One tablet twice a day and finish the course. ● Tell us or your GP before starting any new medicine: this antibiotic interacts with a lot of them. ● It can cause a metallic or altered taste, which settles after the course. ● Seek help the same day if you become breathless, develop chest pain, cough blood, or feel much worse. ● Come back if you are no better 5 to 7 days after finishing.
Follow-up	As Arm 1.

Appendix 1: Observations, thresholds and the CRB score

Every observation below must be measured and recorded before any supply. If ANY threshold is breached, or the CRB score is 1 or more, do not supply: refer.

Oxygen saturation	• Measure SpO2 on air, at rest, after 5 minutes, on a validated pulse oximeter with an adequate trace. • REFER if SpO2 is below 94%. • For a patient with known COPD, refer if SpO2 has fallen below their own documented baseline.
Respiratory rate	• Count for a full 60 seconds. • REFER if 22 or above.
Pulse	• REFER if above 110 at rest.
Blood pressure	• REFER if systolic below 100.
Temperature	• REFER if 38C or above.
Level of consciousness	• REFER on any new confusion, disorientation or drowsiness.
CRB score, WITHOUT the age point	Score 1 point for each of the following. • Confusion: new disorientation in person, place or time. • Respiratory rate 30 or more. • Blood pressure: systolic below 90, or diastolic 60 or less. Score 0: may be treated under this PGD if all other criteria are met. Score 1 or more: do not supply. Refer for same-day assessment. THE AGE POINT IS NOT APPLIED IN THIS SERVICE. The score used here is confusion, respiratory rate and blood pressure only. Why: CRB-65 scores 1 point for being 65 or over. Age 65 and over also qualifies a patient for treatment under this service. A rule requiring a total score of 0 would therefore include every patient in that age group by one criterion and exclude them by the next, and would direct a same-day referral for what may be a well patient with a productive cough, on arithmetic alone. Dropping the age point does not mean treating a 65 year old as a 30 year old. In any patient 65 or over, have a LOWER threshold for referral, take the whole picture into account rather than the numbers alone, and refer if anything about the presentation is not straightforward. Record that you considered it.
Refer regardless of any score	• Focal chest signs suggesting pneumonia. • Haemoptysis. • Cyanosis. • Stridor or absent breath sounds. • Accessory muscle use. • Peripheral oedema with weight gain. • Persistent hoarseness, or a cough over 3 weeks in a smoker. • The patient simply looks unwell to you.

Appendix 2: Key references

- NICE NG120 Cough (acute): antimicrobial prescribing.
- NICE Clinical Knowledge Summaries: Chest infections adult, current revision.
- CRB-65 as described in the BTS community-acquired pneumonia guidance, noting that this PGD deliberately applies it without the age point and states why in Appendix 1.
- Summary of Product Characteristics for doxycycline, amoxicillin and clarithromycin, current versions.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

Version	v004
Supersedes	Version 003, 9 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	
Address	 Postcode:
ODS code, if applicable	

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

[illegible]

Change history

Version	v004
Date	9 September 2026
Changes	• The summary of the governing guidance is restored as part 2 of the document, which is the house format for every Get Real Health PGD: what the PGD is for and which medicines it authorises, then a summary of the guidance that governs the condition, then the PGD itself. Twenty documents lost that section in the September 2026 rewrites, because the generator did not emit it and no check looked for it. It is the section that lets a pharmacist hold the PGD against the guidance it claims to follow, which is exactly how the inverted dental indication should have been caught. It is now a required field: the generator refuses to build a document without it. • THE AGE 65 CONTRADICTION IS RESOLVED. Version 002 listed age 65 and over as a qualifying comorbidity in the inclusion criteria and required a CRB-65 score of 0 in the next bullet. CRB-65 scores 1 point for being 65 or over and a score of 1 was an exclusion, so every patient in that group was included and excluded by consecutive bullets, and was directed to same-day referral for what might be a well patient with a productive cough. The score used in this service is now confusion, respiratory rate and blood pressure only, with the age point deliberately not applied, and the reason stated in Appendix 1. Raised by an adopting pharmacy. • Dropping the age point is paired with an explicit instruction: in any patient 65 or over, have a lower threshold for referral, take the whole picture rather than the numbers, and record that you considered it. The safety net is not simply removed. • ARM 3 NOW STATES ONE RULE. Version 002 stated its eligibility four different ways: the heading said penicillin allergy OR doxycycline unsuitable; the subtitle said doxycycline AND amoxicillin both unsuitable; the scope bullet and the inclusion criteria said doxycycline unsuitable AND penicillin allergy. Two OR-rules, two AND-rules, three different drug pairs. The rule is now written once at the front of the document and repeated verbatim in each arm. • "Doxycycline unsuitable" is reworded as "the first-line agent for this patient is unsuitable", because doxycycline is not first line at 12 to 17 or in pregnancy, so the old wording did not describe those patients. • THE GAP IN ADULT COVER IS CLOSED. A non-pregnant adult for whom doxycycline was unsuitable but who was not penicillin-allergic met the criteria for none of the three arms, although amoxicillin is a NICE first-choice alternative and was already stocked for the 12 to 17 arm. Arm 2 now covers that patient, with the reason doxycycline was unsuitable recorded. • CLARITHROMYCIN AND RENAL FUNCTION. Version 001 halved the dose below a creatinine clearance of 30 mL/min; version 002 did not mention renal function in that arm, so a patient with significant impairment received twice the recommended dose. Version 003 excludes and refers rather than adjusting, consistently with the Skin and Soft Tissue Infection PGD, and requires renal function to be asked about and the answer recorded. • THE SPC AND BNF FAMILIARITY REQUIREMENT IS REINSTATED in all three arms, together with the other standard governance requirements: MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. It had been flagged as missing from the doxycycline arm and was resolved by deleting it from the other two. • Consent in children and young people added: parental responsibility, or Gillick competence with the basis recorded. This PGD covers patients from 12 years. • The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did.

	• The difference from the Skin and Soft Tissue Infection PGD is now stated in the document. Warfarin with doxycycline is a caution here and an exclusion there, with the same practical outcome, and immunosuppression qualifies a patient here while excluding them there. Staff move between these services within a shift and were left to discover the differences. • Records must now carry which arm was used and why, the reason the first-line agent was not used, the CRB score, and where the qualifying comorbidity was age 65 and over, that the lower referral threshold was considered. • Concurrent isotretinoin restored as an exclusion in the doxycycline arm. It was an interaction in version 001, was lost in the version 002 rewrite, and is an exclusion in the doxycycline arm of the Skin and Soft Tissue Infection PGD, so the two documents had diverged on the same drug. Found by comparing against the archived version 001, not by anyone noticing. • Symptomatic relief content is recorded as a deliberate removal. Version 001 carried paracetamol, ibuprofen, codeine linctus and salbutamol advice alongside the antibiotic arms; version 002 made this an antibiotic-only PGD and no change log said so. Analgesia and cough preparations are sold under the pharmacy own protocol, not supplied under this PGD. Erythromycin as an alternative macrolide was dropped at the same time in favour of clarithromycin alone. • Em dashes removed. The document carried five, against a standing house rule, because the generator did not check for them. It now refuses to build a document containing any. • This change history is itemised. Version 002 recorded only "Full clinical review and reissue", which is why an adopting pharmacy had to compare the two versions line by line to find the contradictions above.
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Previous versions

003, 9 September 2026	See the change history of that version. Superseded the same day by v004, which restores the guidance summary section.
002, 7 September 2026	Full clinical review and reissue. Restructured into three arms with observation thresholds and a CRB-65 gate. Introduced the contradictions corrected in 003.
001, earlier 2026	Development and issue of new PGD.